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February 23, 2026

Dr. Thomas Keane
Department of Health and Human Services
Assistant Secretary for Technology Policy
Office of the National Coordinator for Health Information Technology
330 C Street SW
Washington, DC 20201

Attn: HHS Health Sector AI RFI - Request for Information: Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care (Document ID: HHS-ONC-2026-0001-0001, 90 FR 60108)

Submitted electronically via https://www.regulations.gov/document/HHS-ONC-2026-0001-0001

Dear Assistant Secretary Keane,

The National Council of State Boards of Nursing (NCSBN) appreciates the opportunity to submit comment in response to the Department of Health and Human Services' (HHS) request for information "*Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care*."

NCSBN is an independent, non-profit association comprising 59 boards of nursing (BONs) from across the U.S., the District of Columbia, and four U.S. territories. BONs are responsible for protecting the public through regulation of licensure, nursing practice, and discipline of the nearly 6 million registered nurses (RNs), licensed practical/vocational (LPN/VNs), and Advanced Practice Registered Nurses (APRNs) in the U.S. with active licenses.

In this comment NCSBN specifically addresses areas where it believes HHS's RFI on health sector uses of artificial intelligence (AI) can benefit from research exploring ways to mitigate risks associated with AI to prioritize patient safety. This includes examining algorithmic bias in AI systems, transparency and proper oversight of AI systems, and ethical or professional concerns for nurses working in settings with AI technology.

Responses are provided, from a nursing regulation and patient safety perspective, in response to question #9 – *"what concerns do patients and caregivers have related to the adoption and use of AI in clinical care?"*

Algorithmic Bias in AI Systems

As the use of AI technologies becomes increasingly prevalent, policymakers and healthcare providers must take precautions against the risk of algorithmic bias in AI systems. Algorithmic systems can mirror structural data gaps, entrenched biases, and limited knowledge about specific patient populations, which in turn can reproduce or amplify demographic and socioeconomic inequities in healthcare. Algorithmic bias occurs when AI systems inadvertently perpetuate existing healthcare gaps, and if not properly trained

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and validated on varied datasets, the algorithms that run these systems could lead to unequal treatment recommendations or diagnoses.[i]

Often, algorithmic failures are not anomalies but can be symptomatic of broader deficiencies in the design, validation, and deployment of AI-driven technologies that fail to represent a range of patient populations. As noted in a recent Journal of Nursing Regulation article, "disparities highlight the ethical obligation of technology companies to design systems that are inclusive and equitable as a cornerstone of innovation, which will give rise to technologies that contribute to rather than hinder the advancement of optimal human-technology interaction."[ii] Additionally, once AI technologies are deployed, addressing such concerns can be challenging in a healthcare setting, particularly in rural healthcare settings where resource limitations and miscommunications that occur when patients are transferred between facilities contribute to adverse patient outcomes.[iii] These systemic inequities suggest a proactive approach is needed to ensure AI technologies that are deployed mitigate harm and ensure equity rather than exacerbate such concerns.[iv]

A 2023 Journal of the American Medical Association study demonstrates the importance of ensuring AI models are not biased.[v] According to the study, nearly 67% of clinical participants (physicians, nurse practitioners, physician assistants, and others) were not aware AI models could be systematically biased based on patient demographics.[vi] In the study, hospital-based clinicians reviewed clinical vignettes of patients hospitalized with acute respiratory failure, including symptoms, physical examination findings, laboratory results, and chest radiographs. Participants were asked to assess the likelihood of certain diagnoses like pneumonia, heart failure, or chronic obstructive pulmonary disease, with some vignettes supplemented by AI-generated predictions and image-based explanations outlining the model's reasoning. While clinician diagnostic accuracy improved modestly, these gains were undermined when the AI models contained bias. The study found a "systematically biased model decreased accuracy by 11.3 percentage points, which only improved a nonsignificant 2.3 percentage points when the model included explanations for its recommendations." [vii] Notably, the article found AI models offering explanations might not be enough to mitigate harm. These findings suggest that transparency alone may not be enough to counteract embedded algorithmic bias and that reliance on flawed models can meaningfully degrade clinical decision-making and the accuracy and quality of patient care.

Outside of algorithmic biases related to patient demographics, other systemic AI or machine learning biases can include *"label bias"* where a label applied to patients does not mean the same thing for all patients; "*missing data bias"* where missing information affects the predictive value of a model; "*training-serving bias"* where a model is deployed on patients whose data are not similar to the data on which the model was trained; and "*automation bias"* where the provider is unaware that a model is less accurate for a specific group and may trust it too much or rely on inaccurate predictions.[viii] In nursing contexts in particular, a human-centered framework is essential to ensure that technological innovation complements, rather than compromises, professional judgment and patient advocacy. As emphasized in research, "Nurses must be involved in the design, development, and implementation of AI technologies to ensure that these tools align with their clinical expertise and patient-centered values." [ix]

Transparency, Model Limitations, and Oversight of AI Technologies

Research suggests that a lack of transparency in technological systems compounds ethical concerns for clinicians, particularly pertaining to AI-based technologies when clinicians are unable to interpret how decisions are made by 'black box' algorithms.[x] This opacity erodes trust, complicates accountability, and places healthcare providers in ethically uncertain positions when errors occur.[xi] One example that illustrates this concern is an independent review of a sepsis prediction tool in 2021 that illustrated the risks of opaque AI systems in clinical care.[xii] The model was protected as proprietary software and developed using data from only three health systems. There was also limited ability for clinicians and other systems to validate the logic used to compute sepsis alerts.[xiii] When an external validation study was conducted involving nursing informatics experts, physicians, and engineers, that study found the tool had low sensitivity and high alert fatigue.[xiv] The study also noted the lack of publicly available performance data, which left clinicians responsible for outcomes without adequate transparency. Nurses who used the system and other advocates further criticized it for excluding nursing documentation, restricting overrides, and reducing holistic nursing care to fragmented data tasks.[xv]

In addition to issues with transparency, health facilities, providers, and the companies that build AI systems must be vigilant against other model limitations, such as hallucinations or data drift. Independent healthcare research nonprofit and patient safety organization ECRI identified risks with AI-enabled health technologies as the top health technology hazard concern for 2025.[xvi] The report warned that AI systems may contain technical vulnerabilities such as “hallucinations,” in which AI systems generate false or misleading outputs, as well as performance instability resulting from data drift and model “brittleness,” or the inability to adapt appropriately to novel conditions. Further, it cautions that model degradation over time, particularly in systems that continuously ingest new data or operate amid shifting clinical environments, further compounds these risks.[xvii] The report stresses that organizational factors, including insufficient oversight, weak data governance, and overreliance on automated outputs, can heighten patient safety hazards, cautioning that “placing too much trust in an AI model—and failing to appropriately scrutinize its output—may lead to inappropriate patient care decisions.” [xviii]

The Department of Veterans Affairs (VA) Office of Inspector General (OIG) has also recently highlighted the importance of oversight and risk management for AI tools used in clinical care. Last month, the VA OIG released a preliminary result advisory memorandum communicating a potential patient safety risk related to Veterans Health Administration’s (VHA’s) use of generative AI chat tools for clinical care and documentation.[xix] The OIG’s preliminary report expressed that the office found enough concern to issue a preliminary memo to put VHA leadership on notice about AI-related risk pending a final review. The report also generally noted that generative AI can produce inaccurate outputs, including omissions which may affect diagnosis and treatment decisions, and expressed concern about the VHA’s ability to promote and safeguard patient safety without a standardized process for managing AI-related risks.[xx]

These examples highlight how important transparency, understanding technological limitations, and sufficient human oversight are to ensure AI does not undermine the provision of reliable, safe, and accurate patient care. Ensuring these and other safeguards are in place to manage risk should therefore be a high priority for the healthcare tech industry and providers alike.

Ethical and Professional Standards Concerns for Nurses

Nurses constitute the largest segment of the healthcare workforce at nearly 6 million providers nationwide. They are often among the most familiar providers in a patient care team. Given the nursing profession's historically high trust and visibility among patients, patients tend to overwhelmingly hold nurses accountable for decisions made with AI tools.[xxi] From the patient’s vantage point, AI-enabled care may appear streamlined and intuitive; however the unseen side can entail ethical strain and moral conflict

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for nurses, particularly when education, training, and institutional support for AI integration are inconsistent or insufficient.[xxii] As AI tools are increasingly employed in clinical settings, nurses are no longer merely users of technology. They are ethical stewards, patient advocates, and system translators who must bridge algorithmic output and human-centered care.[xxiii]

A nurse's responsibility becomes more complex, not less, as AI takes on more active roles in clinical triage, diagnostics, and risk assessments.[xxiv] Rather than replacing nurses, AI increases the need for clinicians' strong judgment and oversight. Recent research highlights the importance of building critical thinking into the daily practices involving AI's ethical, social, and clinical impacts to prevent dehumanization and depersonalization of care.[xxv] Healthcare organizations can minimize risks by prioritizing comprehensive training, support, and oversight for nurses using AI technologies to ensure nurses have the knowledge and tools to use technology responsibly while also maintaining professional autonomy. Additionally, interdisciplinary collaboration is essential to align technology use with nursing values and legal protections, minimizing the risk of improper use and its consequences.[xxvi] For example, clinical decision tools powered by AI may issue recommendations that conflict with a nurse's professional judgment, creating ethical and legal dilemmas. Nurses may face accusations of negligence or failure to uphold their professional standards if errors occur as a result.[xxvii] For nursing regulators, these situations require both the understanding of systems used and the clinical decision-making of the nurse. Integrating AI technologies into nursing workflows demands technical proficiency and ethical judgment to ensure equitable and patient-centered outcomes. It also warrants recognizing the profession's role as active co-designers and leaders in shaping AI to be consistent with nursing values and priorities.[xxviii]

The Role of the Nursing Regulator

Recognizing the integration of AI into healthcare delivery, it is critical to recognize the role nursing regulators have in the delivery of safe and quality nursing care to the public. BONs can ensure nurses receive proper education about AI in healthcare. Boards of Nursing across the country protect the public through the nursing education program approval and oversight process. Program approval is the official recognition of nursing education programs that meet standards of approval established by Boards of Nursing. The program approval process ensures that programs are qualified to prepare students for licensure and safe and competent practice.[xxix] Nursing regulators could play a role in ensuring the integration of education on AI systems into nursing program curricula. Nursing students graduating with greater AI literacy, understanding how the systems are built and utilized, and the expectations for the nurse will protect patients and strengthen healthcare.

BONs also protect the public through investigating complaints and executing disciplinary processes. As AI continues to play an increasing role in health care, it is critical for regulators to understand how AI was utilized in the delivery of care that led to error and/or patient harm. Increasing transparency into AI systems is vital for nursing regulators to be able to identify potential system issues that may directly impact licensee decision-making. While nurses remain responsible for clinical decisions, there is a need for understanding and regulating the AI systems that may be utilized when those decisions are made.

Conclusion

We appreciate the chance to submit comments in response to this RFI and look forward to opportunities to work with HHS on policies impacting the nursing profession and its regulation. Nursing regulator

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expertise should be sought and included in discussions to ensure patient safety in the use of AI in healthcare delivery. If you have any questions or would like additional information, please do not hesitate to contact Ashley Shelton, NCSBN's Director of Federal and External Affairs (ashelton@ncsbn.org). We look forward to remaining engaged with you on these important issues.

Sincerely,

Ashley N. Shelton, JD
Director, Federal & External Affairs
National Council of State Boards of Nursing

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